Late massive breast implant seroma in postpartum.

PMID: 23846021 | Indexed in PubMed

Late seroma is a rare complication that may occur after a prosthetic breast augmentation. "Seroma" is a generic term used to indicate a serous clear fluid collection, which can develop in surgically dissected areas. A seroma can be defined as "late" if this complication occurs at least 4 months after surgery. Several possible etiologies have been proposed. A 39-year-old old woman with breast implants presented with a huge enlargement of her right breast. Clinical and instrumental evaluation ruled out infection. The swelling was attributed to the presence of fluid adjacent to her implant and aspirated. Nonremission of the fluid collection after aspiration led the authors to surgical removal of the prosthesis, fluid drainage, and capsulectomy. The serous fluid and a portion of the removed capsule was subjected to chemical, cytologic, microbiologic, and anatomopathologic analysis. At the chemical evaluation, the sample of the seroma appeared to be an exudate. Cytologic examination of the fluid showed a large number of neutrophil cells but no malignant cells. Microbiologic evaluation and pathologic findings of the serum sample showed neither the presence of infection nor that of neoplastic infiltration. The postoperative period was uneventful, and the woman experienced no recurrence within 21 months after surgery. This report describes a case of late-onset implant seroma associated with a postpartum breast pump. The authors believe this case could be useful in diagnosing this rare complication and understanding its management. It also may serve to make physicians and nurse practitioners aware of the need for prompt evaluation and treatment.

A Delayed Seroma of a Facial Full-Thickness Skin Graft Donor Site: A Case Report.

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Seromas are characterized as an accumulation of serous fluid beneath the skin, commonly occurring as a postoperative complication. Such formations can occur in dermatologic surgeries where the undermining and dissection of soft tissue create an empty cavity for fluid accumulation. When seromas develop, they usually do so at the wound closure site, within approximately a week after repair. In some cases, seromas have been shown to occur beyond the expected timeframe, coining the term "late seromas." In this context, we present a case of a postoperative seroma noted 21 days after harvesting a full-thickness skin graft from the donor site in the preauricular region.

Persistent Methicillin-Resistant Staphylococcus Aureus Bacteremia Secondary to Infected Seroma: A Rare Case Report.

PMID: 35371705 | Indexed in PubMed

A seroma is defined as a serous fluid collection that develops as a response to injury and surgeries, particularly mastectomy and reconstructive and abdominal surgeries. The majority of the seromas are self-limiting and arise in the acute postoperative period; however, diagnosis of seroma several years after surgery has also been reported in the literature. Persistent bacteremia with infected seroma as a source is a rare entity. We present the first case to be reported of persistent bacteremia secondary to infected seroma with septic emboli to lungs and prostate without any evidence of endocarditis on multiple echocardiograms. This case highlights the importance of meticulous physical examination and source control in the management of bacteremia.

Materials in surgery: a review of biomaterials in postsurgical tissue adhesion and seroma prevention.

PMID: 18816187 | Indexed in PubMed

Postoperative tissue adhesion is a complex inflammatory disorder in which tissues that normally remain separated in the body grow into each other. Seroma is a common postoperative complication that arises when serous fluid collects in the space generated following surgeries that require extensive dissection and that create large empty spaces. Postsurgical tissue adhesion and seroma formation are two serious surgical complications that have received more attention recently from the biomaterials community. This paper provides a review of the pathogenesis and treatment of these surgical complications, with a thorough overview of biomaterial-based treatment and prevention methods.

Giant Perigraft Seroma after Axillobifemoral Bypass for Leriche's Syndrome: A Case Report.

PMID: 27738475 | Indexed in PubMed

Perigraft seroma is a rare complication occurs after placement of any vascular graft. It is defined as the collection of a sterile, clear and acellular liquid around prosthesis. It can appear years after surgery as a soft, palpable and painless mass. We present a perigraft seroma occurred in a 75-years-patient underwent Dacron right axillo-bifemoral bypass for Leriche's syndrome. Ultrasound and computed tomography scan revealed involvement of graft left branch and bifurcation. Although several treatment options have been proposed, removal and replacement of prosthetic affected tract with another of a different material has been proved the choice with best result.

Seromas in the breast: imaging findings.

PMID: 8540992 | Indexed in PubMed

Breast seromas are tumor-like collections of serosanguineous fluid in breast tissue that occur following excisional biopsy, lumpectomy, mastectomy, and plastic surgery procedures such as augmentation, prosthesis explantation, breast reduction, and breast reconstruction. Mammographically seromas are water-density masses located at the surgical site. They exhibit features characteristic of fluid collections on sonographic evaluation. This article reviews the spectrum of imaging findings associated with breast seromas.

Management of abdominal wall recurrent subfascial seroma after pelvic surgery.

PMID: 33525299 | Indexed in PubMed

Seroma is a serous fluid collection that accumulates in dead spaces, where tissue was attached to something before surgery. Abdominal seroma formation is a quite common complication after breast reconstruction with abdominal's flaps or after an abdominoplasty procedure.The most frequently used method for decreasing early seroma frequency are the use of closed suction drains, ultrasonic dissection and sharp dissection, use of fibrine, and use of clip or ligation of vessels during the sur-gery. The management strategies consist of non-operative management, percutaneous drainage, or surgical drainage. With this paper we report a case of a subfascial seroma of the abdominal wall oc-curred in a 41 years old patient after laparotomy surgery for a voluminous pelvic serocele.

Bilateral auricular seromas: a case report and review of the literature.

PMID: 22180116 | Indexed in PubMed

Hematomas, pseudocysts, and seromas are all part of the differential diagnosis of auricular swellings. Seromas are benign collections of serous fluid that have a tendency to recur. The fluid accumulates in the space between the dermis and perichondrium of the ear. We describe what we believe is the first case of spontaneous bilateral auricular seromas to be reported in the literature. One of the seromas resolved in 4 weeks without treatment, and the other resolved with incision and drainage. It is important for physicians to be aware of auricular seromas when considering the differential diagnosis of an auricular swelling, and to understand the various treatment options.

Examination of the Effects of Celecoxib on Postmastectomy Seroma and Wound Healing.

PMID: 32595401 | Indexed in PubMed

To examine the effect of celecoxib on wound healing and development of seroma after mastectomy.Seroma is an accumulation of serous fluid in dead space emerging after breast cancer surgery. The pathophysiology of seroma has not been clearly elucidated. Development of seroma leads to prolongation of hospital stay, increase in costs, ischemia of the flaps, infections due to fluid accumulation, and delayed adjuvant treatment.Seroma is still a current problem, and the most common treatment method for this problem is drainage and repeated aspirations for 5-7 days after surgery. The effect of celecoxib whose anti-inflammatory, antiangiogenic, and antioxidant effectiveness has been demonstrated in a mastectomy model applied on female Wistar rats has been investigated in the present study. A total of 20 rats including 10 rats in the control and 10 in the celecoxib group were studied.Intraperitoneal 0.25 cc/250 g (20 mg/kg/day) celecoxib was administered to the celecoxib group for 5 days after mastectomy, and the same volume of physiological saline solution was given to the control group for 5 days. Rats were followed up for 10 days after surgery. During this process, vitality of the rats, movements of the extremities, wound healing conditions, wound infections, flap necrosis, and occurrence of seroma were recorded. At the end of this period, seromas were aspirated, tissue samples were retrieved, and the rats were sacrificed. Fibrin, hemorrhage, edema, vascularization, congestion, polymorphonuclear leukocytes, and increase in fibrotic tissue fibroblasts, lymphocytes, and macrophages were evaluated in tissue samples.In seroma fluids, interleukin-1 beta (IL-1β), an acute phase reactant, and vascular endothelial growth factor, a vital parameter of vascular proliferation and angiogenesis, were examined. At the end of the experiments, the seroma volume decreased significantly in the celecoxib group ($p=0.804$; 0.001), the IL-1β level decreased significantly as detected in the biochemical examination ($p=0.014$), and in the histopathological examination, an increase in congestion in the celecoxib group was determined. In conclusion, celecoxib markedly decreased interleukin and the volume of seroma after mastectomy; suppressed the level of an acute phase reactant, IL-1β; and demonstrated this effect through its anti-inflammatory activity. We believe that the effects of celecoxib should be investigated using different dose applications and larger number of subjects.

Silver nitrate: a novel therapeutic approach for refractory Seroma following body malodor surgery.

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Seroma, a fluid collection that can develop after surgery, can be a challenging complication to manage. Conventional treatment options, such as quilting suture and drainage tubes, may not be effective in resolving refractory seromas. This article presents two cases of refractory seroma after axillary osmidrosis surgery that were successfully treated with silver nitrate. Silver nitrate, a topical agent with antiseptic, anti-inflammatory, and wound-healing properties, has been shown to be effective in treating perianal fistulas and persistent tracheocutaneous fistulas. In both cases presented here, silver nitrate resulted in complete seroma resolution within 7 and 14 days, respectively. This study suggests that silver nitrate may be a promising treatment option for refractory seroma after axillary osmidrosis surgery. Further research is warranted to validate these findings and establish optimal dosage and treatment protocols.